UPCJ Patient Starter Kit

A printable, fillable companion for your hospital admission and insurance claim. Carry this through every step.

Keep this kit with you from admission to settlement. Every page is numbered with a code such as `UPCJ/SK/1.3/EN` — match it against the journey map on page 4.

Edition First · Version
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How to use this kit

Three rules: carry it, show the right page, keep everything signed.

1. Carry it. Fold it in half along the spine and keep it in the same bag as your insurance card and photo ID.

2. Show the right page. Each guide page has a code (for example `UPCJ/SK/1.3/EN`). That code matches the step you are on in the journey map on page 4. When a clerk asks what you need, show them the page.

3. Keep everything signed. Every template page has a stamp box. If a page you hand to the hospital comes back without a signature or stamp, it is not a valid document. Ask for it to be re-signed before you leave the counter.

What each page looks like

Three page types, each with a fixed layout you can recognise at a glance.

Guide pages — one per step. A black masthead names the step, a grey band shows the phase and path. "Say this" blocks (blue left-border, italic script) are verbatim what to tell the counter. "Check the document has" is the list of fields to verify before you walk away.

Template pages — one per document. Yellow operational note at the top names who fills it. A stamp box (45 mm square) sits bottom-right; a signature rule sits bottom-left. Hand these to the doctor or clerk, then keep the completed page.

Front and back matter — cover (page 1), these two sheets (pages 2–3), decision junction (page 4), glossary and grievance phone tree (page 37).

Which path are you on?

Two questions decide whether you are on the cashless path or the reimbursement path.

Cashless (Path A)

Your hospital is on your insurer's network list AND you intend to use pre-authorised cashless treatment. If both are true, go to step A.1.

Next: step **A.1**.

Reimbursement (Path B)

Your hospital is not in the network OR your cashless request was declined OR you paid out of pocket. Go to step B.1.

Next: step **B.1**.

OPD Consultation

You visit a doctor for the first time about this condition. Everything you do here creates the paper trail your claim will stand on.

What happens

- The doctor examines you and writes a prescription.
- You keep every piece of paper — OPD card, prescription, any referral.
- If the doctor recommends admission, ask for an Admission Advice Note before you leave.

> SAY THIS
>
> *“Please write my diagnosis and the reason for admission on hospital letterhead, with your registration number.”*

Check the document has

- [ ] Patient name
- [ ] Date
- [ ] Diagnosis
- [ ] Medications prescribed
- [ ] Doctor name and registration number

Source: `DOC-003` in the document registry.

Next: **1.2** — Diagnostic Workup.

Diagnostic Workup

You undergo tests ordered by the doctor — blood work, imaging, or pathology — and you must collect the original reports yourself because hospitals will not hand them over unless you ask.

What happens

- The doctor orders investigations: blood tests, X-rays, MRI, CT, or pathology samples.
- Each department — lab, radiology, pathology — produces its own report on hospital letterhead.
- You visit each department's counter and request your original reports before leaving.
- You check that every report carries the lab technician's signature and reference ranges.
- You keep all originals; photocopies are not accepted for reimbursement claims.

> SAY THIS
>
> *“Please give me the original report on hospital letterhead with the lab technician's signature and reference ranges.”*

Check the document has

- [ ] Patient name
- [ ] UHID
- [ ] Date
- [ ] Test name
- [ ] Results
- [ ] Reference ranges
- [ ] Lab technician signature

Source: `DOC-004` in the document registry.

Next: **1.3** — Admission Advice.

Admission Advice

The Admission Advice Note is the trigger document for any insurance claim — without it, neither cashless nor reimbursement can be initiated.

What happens

- The treating doctor writes a formal note advising hospital admission, stating the diagnosis and reason.
- This note must be on hospital letterhead and carry the doctor's name and registration number.
- You hand a copy to the hospital's insurance desk, who use it to begin the claim process.
- Without this note, the TPA cannot open a cashless pre-authorisation or a reimbursement case.
- Get this note before leaving the OPD — once you are admitted, the opportunity may pass.

> SAY THIS
>
> *"Please write the admission advice on hospital letterhead with your name and medical registration number."*

Check the document has

- [ ] Patient name
- [ ] UHID
- [ ] Policy number
- [ ] Diagnosis (ICD-10)
- [ ] Reason for admission
- [ ] Treating doctor name and registration number
- [ ] Estimated duration of stay

Source: DOC-005 in the document registry.

DEADLINE

Before discharge from OPD — Treating doctor must complete this.

Consequence if missed: Neither cashless nor reimbursement can start without this note.

Discharge Summary

The discharge summary is the second most critical document in the entire claim. It must be signed by your treating doctor — not a junior resident — and must include the ICD-10 code for your diagnosis.

What happens

- The treating doctor prepares a summary of your admission, treatment, and discharge.
- This document is used by the TPA to verify the clinical basis of the claim and match it against the pre-auth.
- Do not leave the hospital until you have collected this in your hands.
- Also collect the Final Hospital Bill (Itemised) — DOC-011 — at this step. See the DOC-011 template page for the field checklist.

> SAY THIS
>
> *"Please write the ICD-10 code for my diagnosis and the full reason for admission on the discharge summary."*

Check the document has

- [ ] Patient name
- [ ] UHID
- [ ] Admission date
- [ ] Discharge date
- [ ] Final diagnosis (ICD-10)
- [ ] Procedures performed
- [ ] Medications administered
- [ ] Medications prescribed at discharge
- [ ] Follow-up instructions
- [ ] Treating doctor name and registration number
- [ ] Doctor signature

Source: `DOC-010` in the document registry.

Claim Adjudication

The TPA and insurer review your claim documents and decide the settlement amount. Your role here is mostly to wait — but stay reachable and respond to any queries immediately.

What happens

- The TPA reviews all submitted documents against the policy terms, pre-auth approval, and discharge summary.
- The insurer makes the final settlement decision. The reimbursement settlement SLA is 30 days from complete submission.
- If documents are missing or unclear, the TPA will raise a deficiency query — respond within the deadline stated in the letter.
- Check your claim status weekly by calling the TPA helpline with your claim reference number.

> SAY THIS
>
> *“Please tell me the current status of claim number ___ and whether any documents are pending.”*

Next: **3.3** — Post-Hospitalisation Recovery.

Post-Hospitalisation Recovery

Follow-up consultations, prescribed medicines, and post-discharge lab tests may be claimable within the post-hospitalisation window — typically 60 days, but confirm with your policy, as some specify 60–90 days.

What happens

- Any follow-up consultation, prescribed medication continuing after discharge, or lab test ordered at a follow-up visit is potentially claimable.
- Keep every original receipt — consultation bills, pharmacy receipts, lab reports — along with the corresponding prescription.
- Bills must reference the original hospitalisation to be accepted by the TPA.
- The window is measured from your discharge date. Track the deadline on a calendar.

> SAY THIS
>
> *"I have post-hospitalisation bills from my follow-up within the policy window — please advise how to submit them."*

Check the document has

- [ ] Patient name
- [ ] Date
- [ ] Relation to original hospitalisation
- [ ] Consultation charges
- [ ] Medications prescribed
- [ ] Doctor name

Source: DOC-015 in the document registry.

Next: **3.4** — Final Settlement.

Final Settlement

The insurer issues a Settlement Letter stating the approved amount, deductions, and net payment. If the settlement is correct, the claim closes here. If it is short-paid or denied, you have a structured escalation path.

What happens

- The insurer or TPA issues the Settlement Letter with the approved amount, itemised deductions, co-pay, and net payment.
- Check every line: approved amount versus claimed amount, deduction reasons, co-pay calculation.
- If the settlement is correct and complete, the claim is closed.
- If short-paid or denied, the escalation path is: hospital grievance desk → TPA grievance cell → insurer grievance officer → IRDAI Bima Bharosa (155255 / complaints@irdai.gov.in) → Insurance Ombudsman (zone-wise).

> SAY THIS
>
> *"I would like to register a grievance about claim number ___ — the settlement short-pays the approved amount by ___."*

Check the document has

- [ ] Claim reference number
- [ ] Total claimed amount
- [ ] Approved amount
- [ ] Deductions with reasons
- [ ] Co-pay amount
- [ ] Net settlement amount
- [ ] Settlement date
- [ ] Payment mode and reference

Source: DOC-014 in the document registry.

Pre-Auth Filing

The hospital's insurance desk files a Pre-Authorisation Request Form with the TPA on your behalf. Your job is to hand over the right documents and confirm the form has been submitted.

What happens

- The hospital insurance desk completes the TPA's pre-authorisation form using your policy card, photo ID, and the admission advice note.
- The form must be submitted within 2 hours of the admission advice being issued — delays here delay the entire cashless approval.
- The TPA logs the request and assigns a claim reference number; ask the hospital desk for this number and write it down.
- You do not fill this form — the hospital admin does. But you are responsible for supplying the source documents and for confirming the submission has happened.

> SAY THIS
>
> *"Can you confirm the pre-authorisation has been submitted to the TPA and give me the claim reference number?"*

Check the document has

- [ ] Patient details
- [ ] Policy number
- [ ] TPA ID
- [ ] Admission advice reference
- [ ] Estimated cost breakup
- [ ] Room category
- [ ] Treating doctor details
- [ ] Provisional diagnosis

Source: `DOC-006` in the document registry.

DEADLINE

Same day as admission — Hospital insurance desk must complete this.

Consequence if missed: Cashless may be declined and you will need to pay upfront.

Query / Deficiency Loop

The TPA may raise queries asking for more medical information before approving the pre-auth. The hospital must respond within 24 hours — but you need to actively follow up to keep this moving.

What happens

- The TPA reviews the pre-auth form and may raise a query — called a deficiency — asking for additional reports, doctor notes, or clinical justification.
- The query goes to the hospital, not directly to you. The hospital and treating doctor must respond within 24 hours per the governance SLA.
- If the hospital does not respond in time, the TPA will process the pre-auth with the documents already on file, which can result in a lower sanctioned amount or a rejection.
- Your role is to be the nag: ask the insurance desk at least twice a day whether the TPA has raised any queries and whether those queries have been answered.
- If the pre-auth lapses unanswered, the cashless path may close. Act immediately — do not assume the hospital is handling it.

> SAY THIS
>
> *“Has the TPA raised any queries on our pre-auth? Have those been answered yet?”*

Next: **A.3** — Pre-Auth Approval.

Pre-Auth Approval

The TPA issues a Pre-Auth Approval Letter (AL) confirming the sanctioned amount and any conditions. This is the hospital's green light to treat you on cashless terms.

What happens

- The TPA issues the Approval Letter to the hospital, typically within 4 hours for emergencies and 12 hours for planned admissions.
- The Approval Letter states the sanctioned amount, any conditions or exclusions, the validity period, and whether a co-pay applies.
- The hospital can only begin cashless treatment once this letter is in hand — without it, treatment may be billed to you directly.
- Ask the insurance desk for a copy of the letter for your own records; hospitals do not always volunteer this.
- Check whether the sanctioned amount covers the estimated bill — if not, an Enhancement Request (step A.5) will be needed before your stay ends.

> SAY THIS
>
> *“Please give me a copy of the pre-auth approval letter for my records.”*

Check the document has

- [ ] Sanctioned amount
- [ ] Conditions or exclusions
- [ ] Validity period
- [ ] Co-pay percentage if applicable
- [ ] Sub-limit details

Source: `DOC-007` in the document registry.

Next: **A.4** — IPD Treatment (Cashless).

IPD Treatment (Cashless)

Your inpatient treatment proceeds under the cashless approval. The insurer settles covered charges directly with the hospital — but some items are not covered, and you must pay for those separately.

What happens

- The hospital treats you under the terms of the Approval Letter; covered room, doctor, and procedure charges are billed directly to the insurer.
- Consumables — gloves, syringes, cotton, bandages — are typically not covered by insurance and must be paid by you. Ask the nursing station which consumable charges apply.
- Personal items — food, laundry, attendant charges, phone hire — are not covered. Keep receipts for everything you pay for; these are not claimable but receipts help dispute erroneous charges later.
- Collect the daily treatment chart if you can — photograph the bedside chart each morning. If a bill is disputed at discharge, this is your evidence.
- If your treatment extends beyond the approval validity period or costs rise above the sanctioned amount, the hospital must file an Enhancement Request — see step A.5.

> SAY THIS
>
> *“Can you tell me which charges on today's bill are not covered by insurance so I can pay those separately?”*

Next: **A.5** — Enhancement Request.

Enhancement Request

If the original sanctioned amount is no longer sufficient — because of an extended stay, complications, or additional procedures — the hospital files an Enhancement Request with the TPA.

What happens

- The hospital completes the Enhancement Request Form and resubmits to the TPA with updated cost estimates and medical justification.
- The TPA must respond within 4 hours per the governance SLA; delays here can hold up discharge.
- An enhancement is triggered when: the stay runs longer than the approved validity period, new procedures are added, or total costs are heading above the sanctioned amount.
- You do not file the enhancement — the hospital does. But ask the insurance desk proactively as soon as you know costs may exceed the approval.
- Once the enhancement is approved, the new sanctioned amount replaces the original. Keep a copy of the updated Approval Letter.

> SAY THIS
>
> *"Has the enhancement request been filed with the TPA? What is the revised sanctioned amount?"*

Check the document has

- [ ] Original approval reference
- [ ] Revised estimated cost
- [ ] Reason for enhancement
- [ ] Supporting medical justification

Source: `DOC-008` in the document registry.

Next: **3.1** — Discharge Summary.

Upfront Payment

In the reimbursement path you pay the hospital directly. Every payment must be receipted on a stamped original — these receipts are your claim evidence.

What happens

- You pay all hospital charges out of your own pocket — deposit, interim bills, pharmacy, and the final settlement.
- The hospital issues a receipt for each payment. Insist on stamped original receipts every time. Photocopies fail TPA review.
- Ask for an itemised receipt, not just a payment acknowledgement. The TPA needs to match each receipt to a line item on the hospital bill.
- Keep every receipt in one envelope as you collect them. Do not wait until discharge to ask for duplicates — hospitals often cannot reproduce originals.

> SAY THIS
>
> *“Can I please have an itemised stamped receipt for this payment?”*

Check the document has

- [ ] Patient name
- [ ] UHID
- [ ] Date
- [ ] Amount
- [ ] Mode of payment
- [ ] Receipt number
- [ ] Hospital stamp

Source: `DOC-009` in the document registry.

Next: **B.2** — IPD Treatment (Reimbursement).

IPD Treatment (Reimbursement)

You are receiving treatment and paying for it yourself. Your task during this phase is to preserve every bill and document so the claim file is complete.

What happens

- Treatment proceeds as normal. The clinical care is the same whether your path is cashless or reimbursement — the difference is who pays at each stage.
- You pay out of pocket for the complete treatment: room charges, doctor and surgeon fees, anaesthesia, investigations, pharmacy, consumables.
- Keep every bill, prescription, and treatment record as you go. Do not wait until discharge to collect these — hospital departments do not always retain copies.
- Note which expenses are covered by your policy and which are personal. Non-medical expenses (food, attendant charges, telephone) are typically excluded.
- Photograph every document with your phone as a backup. If an original is lost before submission, a clear photo may help — though the TPA will still require the original.

> SAY THIS
>
> *“Can I have a copy of today's treatment and the corresponding pharmacy bill, stamped by the pharmacy?”*

Next: **B.3** — Records Collection.

Records Collection

Before you leave the hospital, collect the three core documents that form the backbone of your reimbursement claim. Do not leave without all three.

What happens

- This step aggregates three documents from separate hospital departments. You must request each one individually — they are not automatically provided at discharge.
- DOC-010 — Discharge Summary: issued by the treating doctor on hospital letterhead. Must be signed by the treating doctor (not a junior resident). Contains the final diagnosis, all procedures performed, medications administered, and follow-up instructions.
- DOC-011 — Final Hospital Bill (Itemised): issued by the billing department. Must be itemised — room charges, doctor fees, anaesthesia, investigations, pharmacy, and consumables listed separately. If the hospital offers only a summary total, ask explicitly for the itemised breakup.
- DOC-012 — Pharmacy Bills (Stamped): issued by the hospital pharmacy for every medicine dispensed during the stay. Must carry the pharmacy stamp. External pharmacy bills should also be stamped and accompanied by the corresponding prescription.
- Go to the medical records department for the discharge summary, the billing counter for the itemised bill, and the pharmacy for stamped bills. Allow extra time — medical records departments are often busy on discharge day.

> SAY THIS
>
> *"I need the complete medical records file for discharge: the signed discharge summary, the itemised bill, and all stamped pharmacy bills."*

Next: **B.4** — Claim Submission to TPA.

Claim Submission to TPA

You submit the completed claim form and all original supporting documents to your TPA or insurer. This must be done within 30 days of discharge.

What happens

- Download the reimbursement claim form from your insurer's or TPA's website. Fill every field completely — incomplete forms are the most common cause of reimbursement delays.
- Compile all original documents: claim form signed; discharge summary; itemised hospital bill; pharmacy bills; payment receipts; prescriptions; photo ID; policy card.
- Send the full package by registered post or courier. Keep the tracking number and a photocopy of every document you send — originals will not be returned.
- Some TPAs accept walk-in submissions at their office. Call the helpline to confirm the preferred submission method and the exact address.
- Note the claim reference number you receive on submission. You will need this for all follow-up queries.

> SAY THIS
>
> *"I have: claim form signed; discharge summary; itemised bill; pharmacy bills; payment receipts; prescription; ID copies."*

Check the document has

- [ ] Policy number
- [ ] Patient details
- [ ] Hospitalisation details
- [ ] Diagnosis
- [ ] Treating doctor details
- [ ] Total claim amount
- [ ] List of enclosed original documents
- [ ] Bank account details for settlement
- [ ] Patient/policyholder signature

Source: DOC-013 in the document registry.

DEADLINE

Within 30 days of discharge — Patient must complete this.

Consequence if missed: Claim may be time-barred and rejected without review.

Query / Deficiency Loop (Reimbursement)

After reviewing your claim, the TPA may raise queries asking for additional documents or clarifications. You must respond within the stated deadline — typically 30 days.

What happens

- The TPA reviews the submitted claim and may raise a deficiency: a request for missing signatures, additional documents, clarifications on billed items, or proof of payment.
- Deficiency letters are sent to you directly — unlike the cashless path where the hospital manages queries. You are responsible for gathering and submitting the requested items.
- Respond within the TPA's stated SLA, typically 15 days per the governance model. If you do not respond in time, the TPA may process the claim with the documents already on file, which can result in a lower settlement or rejection.
- Send responses by tracked courier. Include a cover letter listing every document enclosed, and reference the query or deficiency number on the letter.
- If the TPA is unresponsive, raises unreasonable queries, or issues an unjust rejection: escalate to IRDAI Bima Bharosa (call 155255) or write to the Insurance Ombudsman for your region.

> SAY THIS
>
> *“I am responding to deficiency notice [reference number]. Enclosed are the requested documents listed in the attached cover letter.”*

Next: **3.2** — Claim Adjudication.

For completion by the Insurer

Insurance Policy Card / E-Card

All fields are mandatory unless marked *optional*.

DOC-001

POLICY NUMBER

TPA NAME AND HELPLINE

SUM INSURED

INR

POLICY PERIOD

POLICYHOLDER NAME

INSURED MEMBERS

Insurer authorised signatory — name & registration number

Source: `DOC-001` in `framework/document-registry.json`.

HOSPITAL STAMP

For completion by the Government

Photo ID (Aadhaar / PAN / Passport)

All fields are mandatory unless marked *optional*.

DOC-002

NAME (MUST MATCH POLICY)

PHOTO

ID NUMBER

Source: `DOC-002` in `framework/document-registry.json`.

HOSPITAL STAMP

For completion by the Doctor

Doctor's Prescription

All fields are mandatory unless marked *optional*.

DOC-003

PATIENT NAME

DATE

DD-MM-YYYY

DIAGNOSIS

MEDICATIONS PRESCRIBED

DOCTOR NAME AND REGISTRATION NUMBER

Treating doctor — name & registration number

HOSPITAL STAMP

Source: `UPCJ/SKD/DOC/003/EN/v1.0/document-registry.json`.

For completion by the Hospital Lab

Lab / Investigation Reports

All fields are mandatory unless marked *optional*.

DOC-004

PATIENT NAME

UHID

DATE

DD-MM-YYYY

TEST NAME

RESULTS

REFERENCE RANGES

LAB TECHNICIAN SIGNATURE

Laboratory in-charge — name & registration number

HOSPITAL STAMP

Source: `DOC-004` in `framework/document-registry.json`.

For completion by the Doctor

Admission Advice Note

All fields are mandatory unless marked *optional*.

DOC-005

PATIENT NAME

UHID

POLICY NUMBER

DIAGNOSIS (ICD-10)

REASON FOR ADMISSION

TREATING DOCTOR NAME AND REGISTRATION NUMBER

ESTIMATED DURATION OF STAY

Treating doctor — name & registration number

HOSPITAL STAMP

Source: DOC-005 Tamil OK / document-registry.json.

For completion by the Hospital Admin

Pre-Authorisation Request Form

All fields are mandatory unless marked *optional*.

DOC-006

PATIENT DETAILS

POLICY NUMBER

TPA ID

ADMISSION ADVICE REFERENCE

ESTIMATED COST BREAKUP

ROOM CATEGORY

TREATING DOCTOR DETAILS

PROVISIONAL DIAGNOSIS

Hospital administrator — name & registration number

HOSPITAL STAMP

Source: docs/DOC-007/Framework/document-registry.json.

For completion by the TPA

Pre-Auth Approval Letter (AL)

All fields are mandatory unless marked *optional*.

DOC-007

SANCTIONED AMOUNT

INR

CONDITIONS OR EXCLUSIONS

VALIDITY PERIOD

CO-PAY PERCENTAGE IF APPLICABLE

SUB-LIMIT DETAILS

TPA authorised signatory — name & registration number

Source: `DOC-007` in `framework/document-registry.json`.

HOSPITAL STAMP

For completion by the Hospital Admin

Enhancement Request Form

All fields are mandatory unless marked *optional*.

DOC-008

ORIGINAL APPROVAL REFERENCE

REVISED ESTIMATED COST

REASON FOR ENHANCEMENT

SUPPORTING MEDICAL JUSTIFICATION

Hospital administrator — name & registration number

Source: `DOC-008` in `framework/document-registry.json`.

HOSPITAL STAMP

For completion by the Hospital

Payment Receipts

All fields are mandatory unless marked *optional*.

DOC-009

PATIENT NAME

UHID

DATE

DD-MM-YYYY

AMOUNT

INR

MODE OF PAYMENT

RECEIPT NUMBER

HOSPITAL STAMP

Hospital authorised signatory — name & registration number

HOSPITAL STAMP

Source: `DOC-009` in `framework/document-registry.json`.

For completion by the Doctor

Discharge Summary

All fields are mandatory unless marked *optional*.

DOC-010

PATIENT NAME

UHID

ADMISSION DATE

DD-MM-YYYY

DISCHARGE DATE

DD-MM-YYYY

FINAL DIAGNOSIS (ICD-10)

PROCEDURES PERFORMED

MEDICATIONS ADMINISTERED

MEDICATIONS PRESCRIBED AT DISCHARGE

FOLLOW-UP INSTRUCTIONS

TREATING DOCTOR NAME AND REGISTRATION NUMBER

DOCTOR SIGNATURE

HOSPITAL STAMP

Treating doctor — name & registration number

Source: doc-011/framework/document-registry.json.

For completion by the Hospital Admin

Final Hospital Bill (Itemised)

All fields are mandatory unless marked *optional*.

DOC-011

PATIENT NAME

UHID

ADMISSION AND DISCHARGE DATES

ROOM CHARGES (PER DAY)

DOCTOR/SURGEON FEES

ANAESTHESIA CHARGES

INVESTIGATION CHARGES (ITEMISED)

PHARMACY CHARGES (ITEMISED)

CONSUMABLES

NURSING CHARGES

TOTAL AMOUNT

INR

AMOUNT PAID BY INSURER (CASHLESS)

INR

AMOUNT PAID BY PATIENT

INR

BALANCE DUE

Hospital administrator — name & registration number

HOSPITAL STAMP

Source: DOC-01 in framework/document-registry.json.

For completion by the Hospital Pharmacy

Pharmacy Bills (Stamped)

All fields are mandatory unless marked *optional*.

DOC-012

PATIENT NAME

DATE

DD-MM-YYYY

MEDICINE NAME

QUANTITY

BATCH NUMBER

AMOUNT

INR

PHARMACY STAMP

Pharmacist — name & registration number

HOSPITAL STAMP

Source: `DOC-012` in `framework/document-registry.json`.
UPCJ/SK/DOC-012/EN v1.0.0

For completion by the Patient

Reimbursement Claim Form

All fields are mandatory unless marked *optional*.

DOC-013

POLICY NUMBER

PATIENT DETAILS

HOSPITALISATION DETAILS

DIAGNOSIS

TREATING DOCTOR DETAILS

TOTAL CLAIM AMOUNT

INR

LIST OF ENCLOSED ORIGINAL DOCUMENTS

BANK ACCOUNT DETAILS FOR SETTLEMENT

PATIENT/POLICYHOLDER SIGNATURE

Patient / policyholder — name &
registration number

HOSPITAL STAMP

Source: DOC-013/Fanwolk/document-registry.json.

For completion by the Insurer

Settlement Letter

All fields are mandatory unless marked *optional*.

DOC-014

CLAIM REFERENCE NUMBER

TOTAL CLAIMED AMOUNT

INR

APPROVED AMOUNT

INR

DEDUCTIONS WITH REASONS

CO-PAY AMOUNT

INR

NET SETTLEMENT AMOUNT

INR

SETTLEMENT DATE

DD-MM-YYYY

PAYMENT MODE AND REFERENCE

Insurer authorised signatory — name & registration number

HOSPITAL STAMP

For completion by the Doctor / Hospital

Follow-up Consultation Bills

All fields are mandatory unless marked *optional*.

DOC-015

PATIENT NAME

DATE

DD-MM-YYYY

RELATION TO ORIGINAL HOSPITALISATION

CONSULTATION CHARGES

MEDICATIONS PRESCRIBED

DOCTOR NAME

Treating doctor / hospital — name & registration number

Source: `DOC-015` in `framework/document-registry.json`.

HOSPITAL STAMP

Glossary & grievance phone tree

Glossary

TPA
: Third-Party Administrator. The intermediary between hospital and insurer that authorises cashless treatment.

ICD-10
: International Classification of Diseases, 10th revision. The diagnosis code the insurer's system requires — see the ICD-10 quick reference (back-matter).

UHID
: Unique Hospital ID. The hospital's identifier for you — appears on every document they generate.

Pre-authorisation
: The TPA's approval to treat you on a cashless basis before admission. Distinct from the final claim settlement.

Sum insured
: The maximum the policy will pay in the policy year across all claims.

Grievance phone tree

1. Hospital grievance desk — file in writing, keep the stamped acknowledgement.
2. TPA grievance cell — call the helpline on `DOC-001`.
3. Insurer grievance officer — contact details in your policy document.
4. IRDAI Bima Bharosa — **155255** or **complaints@irdai.gov.in**.
5. Insurance Ombudsman (zone-wise) — see `cioins.co.in`.